



EVALUATION 4 — SEVERE ACUITY

Primary Diagnosis: Post-Liver Transplant with Immunosuppression, Surgical Wound Complications, New-Onset Diabetes, and Psychiatric Co-Morbidity

PATIENT DEMOGRAPHICS

Field Detail

Patient Name Andrei Vasile Constantinescu

Date of Birth 04/22/1965

Age 60

Sex Male

Race/Ethnicity White / Romanian American

Primary Language English (fluent); Romanian (native — elderly parents in home speak limited English)

Interpreter Needed Yes — Romanian interpreter needed when communicating with parents (they are secondary caregivers)

Marital Status Divorced (2019)

Medical Record Number CI-2025-00439

Address of Service 4715 Eucalyptus Drive, Apt 2C (second floor, elevator building), Menlo Park, CA 94025

Cell Phone (650) 555-0648

Email <andrei.const65@email.com>

County San Mateo

EMERGENCY CONTACTS

Priority Name Relationship Phone Lives with Patient?

1 Elena Constantinescu Mother (age 83, limited English, mild cognitive impairment per family report) (650) 555-0649 Yes — moved in post-transplant

2 Mihai Constantinescu Father (age 86, limited English, mobility impaired — uses walker, hard of hearing) (650) 555-0649 (same line) Yes — moved in post-transplant

3 Irina Petrescu Sister (408) 555-0912 No — San Jose (visits weekends)

4 Stanford Transplant Coordinator Team (650) 555-1600 (24-hour transplant line) N/A

LIVING SITUATION

Field Detail

Lives With Elderly parents (mother 83, father 86) — neither is capable of being a reliable independent caregiver

Residence Type 2-bedroom apartment (2nd floor), elevator access, rented

Stairs None (elevator)

Pets None (immunosuppression — advised against new pets)

DME in Home Hospital bed, bedside commode, rolling walker, wound care supplies (provided by transplant center), home IV pole, digital scale, thermometer, BP monitor, glucometer

Special Considerations Immunocompromised patient — strict infection control required. No fresh flowers, no standing water, no construction/renovation nearby, visitors must be screened for illness.

PAYER INFORMATION

Priority Payer ID / Policy Number
Primary Medicare Part A (ESRD/Transplant) 2NM8-GK4-YP62
Secondary Medicare Part B Same
Tertiary Medi-Cal (SLMB — pays Medicare premiums + limited benefits) 94052817C

PHYSICIAN INFORMATION

Role Name Specialty Phone
Attending / Certifying Dr. Michelle Park Transplant Hepatology (650) 555-1605
Transplant Surgeon Dr. Richard Abramson Transplant Surgery (650) 555-1610
PCP Dr. Omar Farooq Internal Medicine (650) 555-0330
Psychiatrist Dr. Lisa Cho Psychiatry (650) 555-1780
Endocrinologist Dr. Anita Desai Endocrinology (650) 555-1900
Infectious Disease Dr. Christopher Wynn Infectious Disease (650) 555-1655
F2F Encounter Date: 06/08/2025 (inpatient, post-transplant day 19) F2F Performed By: Dr. Michelle Park

REFERRAL / HOSPITALIZATION HISTORY

Event Facility Dates Reason
Liver Transplant Stanford Medical Center 05/20/2025 – 06/10/2025 Orthotopic liver transplant (deceased donor) for ESLD secondary to alcohol-related cirrhosis (MELD 34); complicated by: bile leak (re-operation 05/25), surgical site infection (MRSA — treated), acute rejection episode (treated with pulse steroids), new-onset diabetes after transplant (NODAT)
Inpatient Rehabilitation Stanford Transitional Care 06/10/2025 – 06/18/2025 Post-transplant reconditioning
Pre-Transplant History:

Alcohol-related cirrhosis diagnosed 2020; listed for transplant 2023
Required to complete substance abuse treatment and maintain sobriety x 6 months pre-transplant
Last alcohol use: 11/2023 (per patient report; confirmed by transplant team monitoring)
History of alcohol use disorder, severe (in sustained remission per DSM-5)
History of major depressive disorder, recurrent, severe with prior suicidal ideation (2019 — divorce-related; no attempts)
History of generalized anxiety disorder
Start of Care Date: 06/19/2025

DIAGNOSES

Priority ICD-10 Diagnosis

Primary Z94.4 Liver transplant status (orthotopic, deceased donor, 05/20/2025)

Surgical T86.41 Liver transplant rejection (acute cellular — treated, resolving)

Surgical T81.41XA Infection following a procedure, surgical site, organ and space (MRSA — treated, completing IV antibiotics)

Surgical K83.19 Other obstruction of bile duct (bile leak — repaired 05/25/2025, resolved)

Secondary E13.9 Other specified diabetes mellitus without complications (NODAT — new-onset diabetes after transplant, steroid/tacrolimus-induced)

Secondary Z79.4 Long-term (current) use of insulin

Secondary Z79.899 Other long-term (current) drug therapy — immunosuppressants

Secondary B95.62 MRSA as the cause of diseases classified elsewhere (surgical site — completing treatment)

Secondary F10.21 Alcohol use disorder, in sustained remission

Secondary F33.2 Major depressive disorder, recurrent, severe without psychotic features (current episode — pre-existing, worsened post-transplant)

Secondary F41.1 Generalized anxiety disorder

Secondary K74.60 Unspecified cirrhosis of liver (native liver — removed)

Secondary D64.9 Anemia, unspecified (post-surgical, Hgb 8.9)

Secondary I10 Essential hypertension (new — tacrolimus-related)

Secondary N18.4 Chronic kidney disease, stage 4 (GFR 22 — calcineurin inhibitor nephrotoxicity + pre-existing hepatorenal component)

Secondary E87.1 Hypo-osmolality and hyponatremia (Na 131 at discharge — chronic, monitoring)

Secondary D69.6 Thrombocytopenia, unspecified (platelets 78K at discharge)

ALLERGIES

Allergen Reaction Severity

Sulfa drugs (Bactrim/TMP-SMX) Stevens-Johnson Syndrome (2003) LIFE-THREATENING

Vancomycin Red Man Syndrome (rate-related; tolerated with slow infusion + premedication) Moderate — not a true allergy; requires slow infusion protocol

Shellfish Angioedema Moderate

Latex Contact dermatitis Mild

CURRENT MEDICATIONS (Transplant patients have complex, time-sensitive, interaction-critical regimens)

Medication Dose Route Frequency Purpose Critical Notes

1 Tacrolimus (Prograf) 3 mg PO BID (0800, 2000 — exact 12-hour intervals) Primary immunosuppression HIGH ALERT. Trough levels drawn q Monday AM (target 8-12 ng/mL). Take on empty stomach (1 hr before or 2 hrs after food). NO grapefruit. Drug interactions critical — consult transplant pharmacist before any new medication.

2 Mycophenolate mofetil (CellCept) 1000 mg PO BID (0900, 2100) Immunosuppression (adjunct) Take on empty stomach. Do not crush or open capsules. GI side effects common.

3 Prednisone 15 mg PO Daily AM (tapering: reduce by 2.5 mg every 2 weeks per transplant team) Immunosuppression (steroid taper post-rejection) Monitor glucose closely (steroid-induced diabetes); adrenal suppression risk — do not abruptly discontinue; bone health concern.

4 Daptomycin (Cubicin) 500 mg IV Daily x 14 more days (through 07/03/2025) MRSA surgical site infection (completing course) IV antibiotic — requires PICC line care. Infuse over 30 min. Monitor CPK weekly (rhabdomyolysis risk). Incompatible with dextrose-containing solutions (use NS only).

5 Insulin glargine (Lantus) 18 units SubQ Daily HS (bedtime) NODAT — basal insulin Dose adjustment per sliding scale and endocrinology guidance.

6 Insulin lispro (Humalog) Per sliding scale SubQ Before meals (AC) NODAT — prandial insulin Sliding scale provided by endocrinology. FSBS: QAC + HS (4x/day).

7 Valganciclovir (Valcyte) 450 mg PO Daily CMV prophylaxis (donor CMV+/recipient CMV-: high risk) Monitor CBC (neutropenia risk). Renal dose adjusted for GFR 22.

8 Atovaquone (Mepron) 1500 mg PO Daily with fatty meal PJP (Pneumocystis) prophylaxis (cannot use TMP-SMX — SJS allergy) Suspension — shake well. Must take with food (fat enhances absorption).

9 Fluconazole (Diflucan) 100 mg PO Daily Fungal prophylaxis Drug interaction with tacrolimus (increases tacrolimus levels) — levels being monitored closely.

10 Amlodipine 10 mg PO Daily HTN (tacrolimus-induced) Drug interaction with tacrolimus — dose stabilized.

11 Pantoprazole (Protonix) 40 mg PO Daily (AM, 30 min before food) GI prophylaxis (steroids + immunosuppression)

12 Sertraline (Zoloft) 150 mg PO Daily AM Depression / anxiety Drug interaction note: monitor with tacrolimus; serotonin levels.

13 Lorazepam (Ativan) 0.5 mg PO PRN (max BID) for acute anxiety Anxiety — breakthrough Controlled substance. Monitor for overuse; assess for relapse risk in context of AUD history. Prescriber aware of AUD history.

14 Epoetin alfa (Procrit) 10,000 units SubQ Weekly (Monday) Anemia (CKD + post-surgical) Monitor Hgb weekly; target 10-11 g/dL.

15 Calcium carbonate + Vit D3 600 mg / 400 IU PO BID Bone health (steroid-induced osteoporosis prevention) Take 2 hrs apart from tacrolimus (calcium can affect absorption).

16 Ferrous sulfate 325 mg PO Daily Iron deficiency anemia Take 2 hrs apart from tacrolimus and pantoprazole.

17 Ursodiol (Actigall) 300 mg PO TID Bile duct protection / liver support

18 Magnesium oxide 400 mg PO BID Hypomagnesemia (tacrolimus-induced) Monitor Mg levels.

19 Sodium bicarbonate 650 mg PO TID Metabolic acidosis (CKD) Monitor bicarb levels.

TOTAL MEDICATIONS: 19 Controlled Substances: Lorazepam 0.5 mg (monitored — AUD history) Sharps in Home: Yes — insulin syringes, Procrit syringes PICC Line: Yes — right basilic vein, 4 Fr single-lumen, inserted 06/01/2025, confirmed by CXR tip at cavo-atrial junction. Dressing change: every 7 days and PRN (CHG-impregnated dressing). Flush: NS 10 mL before and after medication; heparin lock per protocol when not in use.

CLINICAL ASSESSMENT — VITAL SIGNS AT ADMISSION

Parameter Value Abnormal?

BP 152/94 Elevated (tacrolimus-related HTN)

HR 92, regular Mildly elevated (anemia, deconditioning)

RR 18 WNL

Temp 99.3°F Borderline — monitor closely in immunosuppressed patient (threshold for concern is lower: contact transplant team if $\geq 100.0^\circ\text{F}$)

SpO2 96% RA WNL

Weight 168 lbs (76.2 kg) — pre-transplant weight 185 lbs Loss of 17 lbs — expected post-transplant

BMI 24.8 WNL

Pain 6/10 (surgical site — right upper quadrant), 3/10 generalized body aches Yes

Blood Glucose 267 mg/dL (fasting) Significantly elevated — NODAT + steroids

WOUND ASSESSMENT

Primary Surgical Wound — Bilateral Subcostal (Chevron) Incision:

Parameter Finding

Location Bilateral subcostal / chevron incision with midline extension to xiphoid

Total Length Approximately 38 cm

Closure Subcuticular absorbable sutures + Dermabond (surgical glue) — majority well-approximated

Complication Area 4 cm segment at right lateral incision line: previously debrided (MRSA infection, 06/02/2025); currently granulating

Wound Bed (complication area) 70% granulation, 30% slough (yellow, adherent, thin)

Dimensions (complication area) 4.0 cm L x 1.2 cm W x 0.8 cm D

Drainage Scant serosanguineous from wound; no purulence since daptomycin initiated

Peri wound Mild induration; erythema resolved (was 3 cm; now <0.5 cm)

MRSA status Most recent wound culture (06/14/2025): no growth. Pending final clearance culture per ID team.

Current Wound Care Orders Cleanse with NS; light debridement of loose slough with gauze (no sharp debridement — thrombocytopenic); apply Aquacel Ag Advantage; ABD pad secondary; Mefix tape. Change every 48 hours and PRN.

Remainder of incision Well-approximated, dry, healing. Steri-strips/Dermabond in place — leave intact until they fall off naturally.

Drain

JP drain removed 06/08/2025 (inpatient).

No active drains.

PICC Site

Parameter Finding

Location Right upper arm, basilic vein

Insertion Date 06/01/2025

Dressing CHG-impregnated transparent dressing (Biopatch + Tegaderm)

Site Assessment Clean, dry, intact. No erythema, swelling, drainage, or tenderness at insertion site.

Line Patency Flushes without resistance; blood return present

Dressing Due Change every 7 days (next due: 06/22/2025)

PICC Complications to Monitor CLABSI (central line-associated bloodstream infection), phlebitis, thrombosis, air embolism, line migration/dislodgement, catheter fracture

HOMEBOUND STATUS

Criterion Finding

Criterion 1 MET. Severe deconditioning post-transplant + prolonged hospitalization (29 days). Ambulates with rolling walker and standby assist, maximum distance 30 feet with fatigue. Surgical pain limits mobility. Additionally, immunosuppressed status makes community outings medically contraindicated — transplant team advises avoiding public spaces, crowds, and contact with potentially infectious individuals during early post-transplant period. Leaving home is a considerable and taxing effort.

Criterion 2 MET. Only planned outings are transplant clinic visits (weekly initially).

Homebound Status CONFIRMED

ADVANCE DIRECTIVES

Document Status

AHCD Yes — executed 05/2025 (pre-transplant). Sister Irina Petrescu designated as agent (not parents due to parents' age/cognition/language).

POLST Yes — Full treatment.

DNR No — Full Code

PSYCHIATRIC / SUBSTANCE ABUSE CONSIDERATIONS

This patient has a documented history of

Alcohol Use Disorder, severe, in sustained remission — last drink 11/2023. Transplant team has a zero-tolerance relapse policy. Patient is monitored by transplant social worker and attends AA meetings (currently via Zoom due to homebound status and immunosuppression). Any evidence of relapse must be reported to the transplant team immediately (may affect transplant follow-up and future candidacy if re-transplant is ever needed).

Major Depressive Disorder, recurrent, severe — pre-existing. Worsened post-transplant (post-surgical pain, deconditioning, financial stress from divorce/job loss, dependence on elderly parents). PHQ-9: 18 (moderately severe). Denies current suicidal ideation — "I just got a new liver, I'm not going to waste it" — but has history of SI (2019). Monitor closely.

Generalized Anxiety Disorder — significant health anxiety post-transplant ("Am I rejecting? Is this infection back?"). GAD-7: 15 (severe).

Lorazepam prescribed PRN — use is being monitored given AUD history. Prescribing psychiatrist (Dr. Cho) is aware and has limited to 0.5 mg BID max with tracking.

Controlled substance monitoring: Lorazepam prescribed in limited quantity (qty 30/month). Wife no longer present for support. Parents unable to effectively monitor. Agency to track utilization and behavioral indicators of potential misuse and coordinate with psychiatrist.

SERVICES ORDERED

Service Discipline Frequency Duration

Skilled Nursing RN Daily x 2 weeks (IV antibiotic + wound care + post-transplant monitoring), then 3x/week x 6 weeks 8 weeks minimum

Physical Therapy PT 3x/week x 8 weeks 8 weeks

Occupational Therapy OT 2x/week x 6 weeks 6 weeks

Medical Social Work MSW 1x/week x 8 weeks 8 weeks

Home Health Aide HHA Daily (personal care, meal prep assist, light housekeeping — essential for immunosuppressed patient's environment)
Duration of need

SKILLED NURSING FOCUS AREAS

Immunosuppressant Management — Tacrolimus level monitoring (trough draws every Monday — fasted, drawn BEFORE morning dose); assess for rejection symptoms (fever, RUQ pain, jaundice, clay-colored stools, dark urine, elevated LFTs, fatigue); assess for toxicity (tremor, headache, GI symptoms, renal function changes). Critical: drug interaction management — NO new medications (Rx, OTC, herbal, supplements) without transplant team approval.

IV Antibiotic Administration — Daptomycin 500 mg IV daily: PICC access, NS flush, infuse over 30 min, flush post-infusion. Monitor for adverse effects (myopathy — CPK weekly; GI symptoms; eosinophilic pneumonia — rare). Assess PICC site per protocol. PICC dressing change weekly (CHG-impregnated). CLABSI bundle adherence.

Wound Care — Chevron incision complication area: wound care per orders every 48 hours. Measure, photograph, document. Assess for MRSA recurrence. Coordinate with transplant surgery and ID.

Diabetes Management (NODAT) — FSBS QAC + HS (4x/day). Insulin glargine and lispro administration/teaching. Sliding scale management. Coordinate with endocrinology for dose adjustments. Steroid taper will significantly affect glucose — expect glucose to decrease as prednisone decreases; risk of hypoglycemia during taper. Teach hypo/hyperglycemia recognition.

Lab Coordination — Weekly: tacrolimus trough, CMP, CBC with diff, Mg, Phos, LFTs, CPK, INR. CMV PCR per transplant schedule. Coordinate all draws with transplant team.

Infection Surveillance — This patient is profoundly immunosuppressed. Temperature check twice daily by patient/parents (contact transplant team if $\geq 100.0^{\circ}\text{F}$ — NOT 100.4°F as in non-immunosuppressed patients; threshold is lower). Assess for opportunistic infections: CMV (fever, GI symptoms, vision changes), PJP (dry cough, dyspnea), fungal (oral thrush, skin lesions), herpes simplex/zoster. Mask and gloves for ALL home visits. Environmental infection control education.

Renal Monitoring — CKD Stage 4 with tacrolimus nephrotoxicity risk. Strict I&O. Daily weight. BMP weekly. Educate on fluid management. Monitor for uremia symptoms.

Psychiatric Monitoring — PHQ-9 every 2 weeks. GAD-7 monthly. Assess medication compliance (sertraline, lorazepam — track lorazepam use). Screen for substance relapse indicators. Coordinate with Dr. Cho (psychiatry) and transplant social worker.

Caregiver Assessment — Parents are unreliable caregivers (age, cognition, language). Sister visits weekends only. Identify and address caregiver gaps. Explore paid caregiver options, adult day health, meal delivery, medication management technology (automated pill dispenser with alarms).

PLAN OF CARE GOALS (60-DAY CERTIFICATION PERIOD)

Goal Target

- 1 No transplant rejection episodes (confirmed by LFTs and tacrolimus levels WNL) 08/17/2025
- 2 MRSA infection resolved; daptomycin completed; PICC removed 07/10/2025
- 3 Surgical wound fully epithelialized 08/17/2025
- 4 FSBS within target range (100-200 fasting, <250 post-prandial) ≥70% of readings 08/17/2025
- 5 Patient demonstrates independent insulin administration and FSBS monitoring 07/15/2025
- 6 Patient verbalizes understanding of all 19 medications (purpose, dose, timing, key interactions, side effects) 07/31/2025
- 7 Tacrolimus levels within target range (8-12) for ≥80% of draws 08/17/2025
- 8 No opportunistic infections 08/17/2025
- 9 Ambulate 200 feet with least restrictive device and standby assist 08/17/2025
- 10 Independent with ADLs (bathing, dressing, grooming, toileting) 08/01/2025
- 11 PHQ-9 decreased to ≤14 (from 18) 08/17/2025
- 12 Zero substance relapse 08/17/2025
- 13 Sustainable caregiver support plan in place 07/31/2025
- 14 Zero falls, zero hospital readmissions 08/17/2025
- 15 Renal function stable (GFR >20) 08/17/2025

PATIENT SIGNATURES — EVALUATION 4

Patient Signature: _____ Date: _____

Print Name: Andrei Vasile Constantinescu

Clinician Signature: _____ Date: _____

Patient / Representative Signature

Date

RN Clinician Signature

Date / Time